

Paediatrics

The curriculum in pediatrics, 2002 has been revised and updated in 2012 to emphasize the issues related to child health problems of the country.

The undergraduate medical students need to know these common childhood problems and how to manage these efficiently. This need based revised curriculum will certainly enable them to serve the community.

The contents of the curriculum as well as the skills to be acquired by the students are categorized as “must know”, “useful to know”, “nice to know” according to their importance at this level. These categories are marked as ***, ** and * respectively. Teachers are requested to follow this guideline while planning their teaching-learning sessions.

Departmental Objective:

To train medical graduates who will be able to manage common childhood problems in the community. Hence, at the end of the course they will be able to –

- manage common pediatric and neonatal problems at hospital and the community level.
- manage acute neonatal and pediatric emergencies efficiently
- identify neonatal and pediatric problems that require secondary and tertiary care and refer them appropriately.
- To diagnosis and manage pediatric emergencies commonly encountered in hospital practice.
- refer appropriately for rehabilitation where necessary
- use growth chart in order to assess the growth of a child to differentiate normal from abnormal.
- provide emergency cardiopulmonary resuscitation to newborns and children
- select and interpret relevant investigations
- perform routine therapeutic procedures
- communicate effectively with the child, parents, relatives and colleagues.
- counsel, explain and guide parents and relatives regarding the illness, the management plan, the possible complications and the prognosis
- participate in the national programmes providing both service and training and preventive activities: IMCI, NNS, EPI and other programmes
- serve the community during disaster and epidemics
- update with latest information related to core paediatric problems
- conduct research
- perform/discharge medico-legal and ethical responsibilities

List of Competencies to acquire:

- communicate and counsel patients, parents and relatives.***
- demonstrate empathy and humane approach towards patients, parents and relatives. ***
- exhibit a proper attitude towards colleagues and other staffs.***
- take relevant history and perform clinical examination to arrive at a working diagnosis***
- perform the anthropometric measurements in order to assess the growth of a child.***

- use and interpret the growth chart to compare the anthropometric values with the standard one.***
- suggest appropriate investigations keeping in mind their relevance and cost effectiveness***
- plan and outline a treatment at primary facilities which is need based, cost effective and evidence based***
- recognize situations which need urgent treatment at secondary and tertiary level hospitals and be able to make a prompt referral with a referral note after giving first aid or emergency treatment at primary health care facilities.***
- use and interpret the Integrated Management of Childhood Illness (IMCI) Chart prepared by WHO***
- prepare and administer oral rehydration therapy (ORT)***
- explain mother about appropriate positioning and attachment in breast feeding & effective suckling**

Students must observe the following skills

- Hand/ forearm washing***
- Cardio-pulmonary resuscitation (CPR)***
- First aid to children and neonates including endotracheal intubation and mouth to mouth breathing.**
- Lumbar puncture***
- Bone marrow aspiration***
- Thoracocentesis/ paracentesis*
- Umbilical catheterization*
- Exchange transfusion*
- Blood and blood products transfusion including mobile transfusion***
- I/V cannulation, collection of samples for routine examination (RE)*
- Use of AMBU bag***
- Administration of an enema*
- Phototherapy**
- Incubator (open and closed) care*
- Oxygen therapy***
- Nebulization***
- Bedside urine for albumin & sugar***
- Capillary blood glucose estimation**
- Preparing balanced diet**
- Performing intradermal / subcutaneous/ intramuscular/intravenous or per rectal injections in children*
- Constructing a vaccination schedule for a child*
- Applying vaccine to children*
- Mantoux test and interpret the result*
- Introduction of nasogastric tube*
- Managing hyperpyrexia or hypothermia and convulsion and other paediatric emergencies*

- Applying otoscope, tongue depressor during examination of the child*
- Writing discharge certificate*

Final Professional Examination:

Marks distribution:

Total marks – 500 (Summative)

Pediatrics -130

Written = 50

- MCQ-MTF (05) + SBA(05)=10
- 2SEQ 20-+ 6SAQ 15 = 35
- Formative assessment = 05

Oral, Practical and Clinical=80

- Oral, Practical =30 +10 =40
- Clinical-1 long case-20
- -1 Short case-10
- OSPE =10.

Components	Marks			Total Marks
Paper I – Internal Medicine				100
Paper - II - Medicine with allied and Pediatrics	Int Me.& Allied	Pediatrics		
Pediatrics MCQ (MTF, 5 + SBA, 5)	10	10	20	
Total 2 SEQ + 6SAQ Group B1 - 1SEQ 10+3SAQ (2.5X3) Group B2 – 1SEQ10 +3SAQ(2.5X3)	35	35	70	
Formative assessment	05	05	10	
	Total			
OSPE		10		
Oral, Practical and clinical		30+10+30		
	Total	130 (For Pediatrics)		

Paediatrics

Learning Objectives	Contents	Teaching Hours
At the end of the sessions, students will be able to - define Pediatrics and Primary health care - state the stages of a child's life - describe the current child health status in Bangladesh - describe the major child health problems in the country - describe Millennium Developmental Goals (MDG), particularly MDG 4 - describe the components of essential service package (ESP) and essential newborn care (ENC) - discuss the emergency triage assessment and treatment - state the National Child Health programmes - describe the preventive programmes of pediatrics e.g. Integrated Management of Childhood Illness (IMCI), EPI, National Nutrition Services (NNS), Infant and Young Child Feeding (IYCF), vitamin-A supplementation	Preventive Paediatrics CORE: - An introduction to Paediatrics & MDG*** - IMCI*** - EPI*** - IYCF*** - IDD** - ENC** - NNS*** - ETAT** - ECD** - Vitamin-A supplementation**	1 hr 2 hrs 1hr Total = 4 hrs

Learning Objectives	Contents	Teaching Hours
At the end of the sessions the students will be able to • define Infant and young child feeding (IYCF) • describe IYCF global & national perspectives and IYCF recommendations • describe the effective breast feeding; exclusive breast feeding (including colostrum) • describe advantages of breastfeeding and hazards of artificial feeding • describe anatomy of breast and physiology of lactation • describe techniques of breastfeeding: position and attachment & effective suckling • counsel for breast feeding & complimentary feeding • describe the baby friendly hospital initiatives • describe breast milk substitute (BMS) code • describe maternal nutrition & drugs in breastfed mother • describe guiding principle of complementary feeding & advantage of complementary feeding, age specific appropriate food	Infant and young child feeding (IYCF) <u>CORE:</u> • Breast feeding*** • Complementary feeding***	1 hr 1hr Total = 2 hrs

Learning Objectives	Contents	Teaching Hours
At the end of the sessions the students will be able to - define diarrhoea, it's aetio-pathogenesis, classification, clinical presentation, complications of diarrhoea - define persistent diarrhoea and dysentery - assess dehydration & to offer appropriate management (Plan A, B,C) - select relevant investigations and their interpretation - describe the composition of ORS, Cholera Saline, Ringer's solution. - describe prevention of diarrhoea - describe helminthiasis and their management	Gastrointestinal disorders CORE: - Diarrhoeal disorders & management*** - Acute watery diarrhoea*** - Dysentery*** - Persistent diarrhoea*** - Abdominal Pain & Helminthiasis**	1 hr 1 hr Total = 2 hrs
At the end of the sessions the students will be able to - state the common respiratory illnesses of children - describe aetiology, clinical presentation, complication & management of pneumonia - describe aetiology, clinical presentation, complication & management of bronchiolitis - state the common causes of respiratory distress - differentiate asthma, pneumonia and bronchiolitis - define childhood asthma & describe the presentation & management of asthma. - describe the common differential diagnoses of stridor in children - describe the management of a case of acute laryngotracheobronchitis	Respiratory Disorders CORE: - ARI*** - Pneumonia*** - Bronchiolitis*** - Childhood Asthma*** - Croup and other causes of stridor And their management**	1 hr 1hr 1hr Total = 3 hrs

Learning Objectives	Contents	Teaching Hours
At the end of the sessions the students will be able to • list the common causes of anaemia in children • classify anaemia. • describe the risk factors, clinical presentation & management of iron deficiency anaemia. • describe the pathogenesis, clinical & laboratory features and management of congenital haemolytic anaemia (CHA) • differentiate the laboratory features of these 2 diseases • counsel the parents about the prognosis of CHA. • describe the cause/ differential diagnoses of bleeding disorder. • describe the etiopathogenesis, clinical presentations, laboratory features and management of ITP, hemophilia, von Willebrand disease and aplastic anaemia	Haematological Disorders <u>CORE:</u> • Iron deficiency anaemia*** • ITP *** • Haemophilia*** • Congenital haemolytic anaemia *** • Hypoplastic anaemia/ aplastic anaemia**	1hr 1 hr Total = 2 hrs

Learning Objectives	Contents	Teaching Hours
At the end of the sessions the students will be able to - list the common causes of generalized swelling and haematuria among children - define and classify nephrotic syndrome - describe the aetio-pathology, cardinal features, complication, diagnosis, treatment and prognosis of nephrotic syndrome. - describe aetio-pathogenesis of acute glomerulonephritis, clinical presentation, complication & management of acute glomerulonephritis. - identify & describe management of a child with hypertensive encephalopathy & acute LVF - differentiate nephrotic syndrome from acute glomerulonephritis - describe the aetiology, risk factors, pathogenesis, cardinal features, complications, laboratory findings & management of UTI in children - counsel the parent for prevention of UTI - describe the causes, clinical presentation, complication & management of acute renal failure - describe the fluid & electrolytes homeostasis and acid base homeostasis - name common fluid, electrolytes and describe acid base imbalance.	Renal disorder CORE: - Nephrotic syndrome*** - Acute glomerulonephritis*** Acute Renal Failure** Fluid & Electrolytes & acid base balance*** - Urinary Tract Infection***	1 hr 1 hr 1hr Total = 3hrs

Learning Objectives	Contents	Teaching Hours
At the end of the sessions the students will be able to - state the different causes of jaundice - describe the clinico-pathological consequences of hepatotropic viruses - describe the aetiopathogenesis, clinical presentation and complications of acute hepatitis - describe the stigmata of chronic liver diseases (CLD)/ cirrhosis of liver - list the relevant investigations for a child with liver disease e.g. acute hepatitis or chronic liver disease etc and their interpretation. - describe the treatment of a child with acute hepatitis or chronic liver diseases - describe the clinical presentation & management of hepatic coma. - list the common causes of haematemesis in children - describe the aetio-pathogenesis, clinical presentation of a case of portal hypertension. - outline the management of a case of haematemesis and melaena	Diseases of Liver <u>CORE:</u> - Viral hepatitis *** - Fulminant hepatic failure*** - Hepatic coma/ hepatic encephalopathy*** - Portal hypertension ** - Chronic liver disease eg. cirrhosis**	1 hr 1 hr Total = 2 hrs

Learning Objectives	Contents	Teaching Hours
At the end of the sessions the students will be able to • classify congenital heart diseases • describe the haemodynamics, clinical presentation, complication & management of common congenital heart diseases e.g. ASD, VSD, TOF & PDA. • describe aetio- pathogenesis of acute rheumatic fever • describe the clinical presentation, diagnosis, & management of acute rheumatic fever and rheumatic carditis. • describe the prevention of acute rheumatic fever • describe the causes, clinical presentation & management of heart failure in infant & children	Disease of Cardio-vascular system <u>CORE:</u> • Congenital heart disease (ASD, VSD, TOF & PDA)*** • Rheumatic fever & Rheumatic heart disease*** • Heart failure in infancy & childhood***	2 hrs 1 hr Total = 3 hrs

Learning Objectives	Contents	Teaching Hours
At the end of the sessions the students will be able to • describe causes of convulsions in children • describe the criteria of diagnosis & management of febrile convulsion • describe the aetio-pathogenesis, clinical presentation & management & prognosis of acute pyogenic and viral meningitis • describe the aetio-pathogenesis, clinical presentation & management & prognosis of encephalitis • describe the pathogenesis, clinical staging, management & prognosis of tubercular meningitis. • describe the CSF findings of acute bacterial, tubercular and viral meningitis • define and classify epilepsy • describe the clinical presentation, management & prognosis of epilepsy • define and list the differential diagnoses of acute flaccid paralysis (AFP). • describe the clinical presentation, management & complication of GuillainBarre syndrome (GBS), poliomyelitis and transverse myelitis • differentiate GBS, polio and transverse myelitis • describe causes of mental retardation, its management, counseling & rehabilitation • define cerebral palsy & describe its causes, types, clinical feature, management, counseling & rehabilitation	Disease of Nervous system <u>CORE:</u> • Febrile convulsion *** • Epilepsy** • Meningitis & Encephalitis • Mental retardation ** • Cerebral palsy** • Acute Flaccid Paralysis (AFP)*** • GuillainBarre syndrome • Transverse myelitis • Polio myelitis	1hr 1hr Total = 2 hrs

Learning Objectives	Contents	Teaching Hours
At the end of the sessions the students will be able to - Enumerate common childhood malignancies - define and classify leukaemia - describe the clinical presentation and diagnosis of acute leukaemia - describe the blood & bone marrow features of acute leukemia - describe the treatment of acute leukaemia - classify lymphoma	Malignant diseases CORE: - Leukaemia*** - Lymphoma & other tumours*	1 hr
At the end of the sessions the students will be able to - describe the causes of short stature - describe the aetiopathology, clinical presentation, diagnosis & management of hypothyroidism - classify diabetes mellitus & describe the clinical presentation, diagnosis & management of type I (IDDM) Diabetes Mellitus - classify the chromosomal disorders - describe clinical presentation, management and prognosis of Down syndrome - counsel parents about the prognosis of the diseases mentioned above	Endocrine and Chromosomal Disorders CORE: - Short stature *** - Hypothyroidism*** - Diabetes Mellitus * - Down syndrome***	1 hr 1hr Total = 2 hrs

Learning Objectives	Contents	Teaching Hours
At the end of the sessions the students will be able to • list the common causes of pain and swelling of joints • classify juvenile idiopathic arthritis (JIA) • describe clinical manifestations and complications of JIA. • describe relevant investigation and interpretation • enumerate the different treatment options of JIA • classify myopathy • describe the clinical features and diagnosis of pseudo hypertrophic muscular dystrophy/ Duchene muscular dystrophy (DMD) • describe the relevant investigations and their interpretation • describe the management including counseling & rehabilitation of pseudo hypertrophic muscular dystrophy (DMD)	Connective Tissue & Musculo-skeletal Disorders <u>CORE:</u> • Juvenile idiopathic arthritis (JIA)*** • Myopathy • Pseudohypertrophic muscular dystrophy**	1 hr
At the end of the sessions the students will be able to • list the common accidents and emergencies of children • describe the principles and management of poisoning • describe the clinical presentation, complications and management of kerosene poisoning • describe the clinical presentation, complications and management of organophosphorus poisoning • describe the aetio-pathogenesis, clinical presentation and management of snake bite • describe the pathogenesis and clinical presentation of drowning (salt and fresh water drowning)	Accidental poisoning & Drowning <u>CORE:</u> • Kerosene*** • Organophosphorus compound*** • Snake bite** • Drowning**	1 hr 1hr Total = 2 hrs

Learning Objectives	Contents	Teaching Hours
At the end of the sessions the students will be able to - state the common behavioral disorders of children - describe the risk factors & management of nocturnal enuresis - differentiate true seizure from pseudo-seizure - describe causes, early identification management & counseling of autism spectrum disorder (ASD) - describe child abuse and neglect	Paediatric Psychological and Psychiatric disorder <u>CORE:</u> - Childhood behavioural disorders** - Autism spectrum disorder (ASD)*** - Somatoform disorder** - Enuresis*	1 hr
At the end of the sessions the students will be able to - describe the steps of communication /counseling - counsel a parent or care giver regarding any illness	Communication & Counseling <u>CORE:</u> Counseling	1 hr

Pediatrics

Teaching/ Learning Methods & Aids

Teaching methods	Aids
Lectures: • Large group teaching & lectures • Small Group teaching: (Clinical) ▪ Bedside teaching ▪ Case demonstration & practice ▪ Practical Skills (Video) • Field Site training : (with Community Medicine) • Integrated Teaching • Self-directed learning	OHP/ Multimedia presentation, Video, Slide • Patients • Simulated Patients • Dummy (Manikins) • Charts e.g. growth chart, IMCI Chart • Reading materials ○ Modules & national guidelines on different childhood illnesses ○ Study guide ○ Books, journals • Others e.g. ECG, Instruments, X-ray, photographs

ACADEMIC CALENDAR – PAEDIATRICS

LECTURE	2 nd Phase			3 rd Phase	4 th Phase / Final Phase			
	4 hours			20 hours	26 hours			
	INTRODUCTION PREVENTIVE PAEDIATRICS			IYCF, Growth & development, Nutritional disorders, Infectious diseases, Childhood tuberculosis, Respiratory disorders, Gastrointestinal disorders, Accidental poisoning	Neonatology, Hematologic disorders, Renal disorders, Disease of liver, Disease of cardiovascular system, Diseases of nervous system, Malignant diseases, Endocrine and chromosomal disorders, Connective tissue & musculoskeletal disorders, , Paediatric Psychological and Psychiatric disorders, Communication and counseling			
CLINICAL	4 weeks				6 weeks			
	2 WEEKS		2 WEEKS	No clinical placement in 4 th year	INDOOR PLACEMENT			
	Day	IMCI	Neonatology		Morning (2 hours)	Evening (2 hours)		
	1	IMCI	History writing		1st Week D1-2 : Introduction + history taking D3 : IMCI D4-5 : Cough & difficult breathing, diarrhea D6 : Presentation & discussion 2nd Week D1 : Bleeding disorder D2 : Pallor D3-4 : Fever, Leukaemia D5 : Accidental poisoning D6 : Presentation & discussion 3rd Week D1- 2: PEM D3-4: Hepatosplenomegaly D5 : Lymphadenopathy D6 : Presentation & discussion 4th Week D1- 3: Scanty urine, ARF, NS/AGN D4 : RF & RHD D5 : Joint swelling D6 : Presentation & discussion 5th Week D1-4 : Neonatology D5 : IYCF D6 : Presentation & discussion 6th Week D1-2: Convulsion D3 : Developmental Assessment D4- 5: OSCE D6- : Feedback	Self-directed learning Self-directed learning Self-directed learning Self-directed learning Self-directed learning Self-directed learning Self-directed learning Self-directed learning Self-directed learning Self-directed learning Self-directed learning		
	2	IMCI	Clinical examination of i. Newborn ii. Child				Self-directed learning Self-directed learning Self-directed learning Self-directed learning Self-directed learning Self-directed learning Self-directed learning Self-directed learning	
	3	IMCI						
	4	IMCI						
	5	IMCI	Common neonatal problems: • Perinatal asphyxia • Low birth weight • Neonatal sepsis • Neonatal Jaundice • Neonatal convulsion					Self-directed learning Self-directed learning Self-directed learning Self-directed learning
	6	IMCI						
	7	IMCI						
	8	IMCI						
	9	IMCI	IYCF					Self-directed learning Self-directed learning Self-directed learning
	10	IMCI						
	11	Assessment			Assessment	Self-directed learning Self-directed learning		
	12	Feedback	Feedback					

PLAN FOR ACADEMIC CALENDAR – PAEDIATRICS

Annex-

FIRST PROF.

SECOND PROF.

THIRD PROF.

FINALPROF.

6m	6m	6m	6m	6m	6m	6m	6m	6m	6m
			4 LECTURE		20 LECTURE		22 LECTURE		
			Introduction to Pediatrics, MDG -1 IMCI-2 National programmes-1		IYCF-2 (breast feeding-1, complementary feeding-1) Growth & development-2 Protein energy malnutrition, SAM, CMAM- 1 Other Nutritional disorders -1 Infectious diseases -7 Respiratory disorders- 3 Gastrointestinal disorders -2 Accidental Poisoning -- 2		Neonatology – 4 Haematologic disorders – 2 Renal disorders – 3 Disease of liver – 3 Disease of cardiovascular system – 2 Disease of nervous system – 2 Malignant disease – 1 Endocrine and chromosomal disorders – 2 Musculoskeletal disorders -- 1 Pediatric psychological and psychiatric disorders – 1 Communication and counseling -- 1		
			CLINICAL				CLINICAL		
			4 WEEKS				6 WEEKS		10 days for block teaching
0	Yr -1		3 rd	Yr -2	4 th	Yr -3	5 th	Yr -4	Yr -5

Photograph

Name :

Session :.....**Batch** :..... **Roll Number** :.....

Group : **Phase II** :.....**Phase IV**.....

Period of attachment :

Phase II :**Phase IV** :.....

Contact address with phone No :

NOTE:

- Students must complete the activities shown on the card during the clinical attachment in Paediatrics.
- Card will be signed by registrar grade and above.
- At the end of the attachment the card must be submitted and signed by the Head of Department. The card will be retained by the Department.
- During 2nd round, students have to write down history, to perform physical examination, to observe the management and follow-up including counseling in two of their allocated beds.
- Each student will submit five complete case history.
- At the end of each phase formative assessment will take place and marks of formative assessment will be added to the summative assessment.
- Ward duties will start from 09:30 am to 11:30 am & from 06:00 pm to 08:00 pm (total 04 hours) in each day.

Summative assessment of Paediatrics

Assessment system and mark distribution:

Components	Marks
Formative assessment	5
Paper – II Paediatrics Written (Group B1 and B2) MCQ (Single based answer + Multiple True False) SEQ (2) + SAQ (6)	10(5+ 5) 35
OSPE	10(5+5)
Oral & Practical	40(30+10)
Clinical: 1 Long case 1 Short case	20 10
Grand Total	130

Pass mark will be 60% in each written, oral, practical & clinical examination

Prerequisite for appearing in Final Professional examination for Paediatrics

After successful completion of Lectures, clinical placement, Integrated teaching & Block posting students will appear in final professional examination. Eligibility for final professional examination is subjected to

- 75% attendance in Lectures and integrated teaching.
- 75% attendance in Clinical placement and block posting.
- 60% marks in Formative assessment.

1st Round (2nd Phase MBBS) Duration – 4 weeks (96 hours)

Learning Objectives:

The student will be able to describe

- describe the definition of paediatrics
- Who is a child? Stages of a child's life
- The current child health statistics e.g. NMR, IMR, under 5 mortality etc.
- Definition and important components of MDG and SDG
- IMCI strategy, the principles of integrated care, IMCI case management process
- Major health problem in paediatrics
- Develop interpersonal and communication skills benefiting a physician in order to discuss illness and its outcome with patient and family.
- Different components of paediatric history – particulars of the patient, presenting symptoms, history of the present illness, history of past illness, birth history, feeding history, immunization history, developmental history, treatment history, family history, personal & social history etc.
- Perform clinical examination and will be able to elicit different signs.
- National child health programme- IMCI, IYCF, EPI, CNCP, ETAT etc.

Time Management :

2nd Phase = 4 weeks

6 days / week, 24 days in Total

32 hours in morning

32 hours in evening

16 hours in Outpatient Department

16 hours in Emergency Department

Duration of Placement (1stRound) fromto

Total attendancedays, out ofdays

SL	Date	Topic(morning) 9.30- 11.30 am	Teac her's initial	Topic (Evening) 6 - 8 pm	Teac her's initi al
1		• Introduction to Paediatrics. • Introduction of IMCI. • Introduction of IMCI student's hand book • Introduction of IMCI Wall Chart, case recording form • Reading on introduction of • General danger signs, cough or difficult breathing		• Reading on Introduction, General danger sign, cough & difficult breathing • Practice on relevant cases	
2		• Video exercise on general danger sign, cough & difficult breathing • Case demonstration • Clinical practice by the students (up to cough & difficult breathing) • Reading on diarrhea		• Reading on diarrhea • Practice on relevant cases	
3		• Video exercise on diarrhea & dehydration • Case demonstration on diarrhoea • Clinical practice by the students upto diarrhoea • Reading on fever and measles		• Reading on fever and Measles • Practice on relevant cases	
4		• Video exercise on fever & measles • Case demonstration on fever & measles • Clinical practice by the students upto fever & measles • Reading on ear problem & checking nutritional status: malnutrition & anaemia		• Reading on ear problem & checking nutritional status: malnutrition & anaemia • Practice on relevant cases up to fever	
5		• Video on ear problem, malnutrition & anaemia • Demonstration of WHO growth charts • Case demonstration on		• Reading on immunization status, assessing the child's feeding up to other problems • Practice on relevant cases	

		- malnutrition - Reading on immunization status, assessing the child's feeding up to other problems			
6		- Clinical practice on full assessment by the student - Drill on fast breathing - Reading Identify treatment & treat the child		Reading on identify treatment & treat the child	
7		- Reading on counseling & follow-up - Introduction of backside of case recording form - Clinical practice on full assessment by the students including the backside		- Reading on counseling & follow-up - Practice on relevant cases	
8		- Role play on treat the child, demonstration & practice by students - Reading on sick young infant - Introduction of case recording form of sick young infant		Reading on sick young Infant	
9		- Video on sick young infant & feeding assessment (Positioning & attachment) - Case demonstration on sick young infant - Clinical practice by the student on sick young infant		Practice on full assessment of the students including back side	
10		- Demonstration on feeding assessment (Positioning & attachment) - Clinical practice by the students on feeding assessment (Positioning & attachment) in the postnatal ward - Drill on weight for age - Review & feed back		- Reading on infant & young child and early childhood development - Review	
11		Newborn Resuscitation		History Taking	
12		Low Birth weight		General Examination	
13		Neonatal Jaundice		Examination of GIT	

14		• Examination of Respiratory System		• Examination of CVS	
15		• Examination of Nervous System		• Practice on relevant cases	
16		• Examination of Musculo Skeletal System		• Practice on relevant cases	
17		• Assessment by OSPE+ MCQ+SAQ			
18		• Feedback with all faculty members			

Marks Obtained (%):

Comment:

GRADING

A = 75-100%

B = 60-74%

C = 50-59%

D = 40-49%

E = 00-39%

Professor

Department of Paediatrics

Registrar

Department of Paediatrics

2nd Round (4th Phase MBBS) **Duration – 06 weeks (144 hours)**

Learning Objectives:

At the end of round students will be able to-

- develop skills in history taking & physical examination.
- identify sign & symptom of different systems.
- Interpret the findings in terms of diseases, make differential diagnosis & an laboratory investigations.
- Identify instruments commonly used for medical procedures and observe the doctor performing the procedures.
- assess the growth and development of the child and early childhood development(ECD).
- know different nutritional disorders.
- Know the infectious diseases.
- know common neonatal problems.
- diagnose and manage diseases of different systems given below:
Alimentary tract, Liver, Biliary tract and pancreatic disease
Cardiovascular disease
Respiratory Disease
Kidney and Genito-urinary disease
Neurological diseases

Blood disorders
Musculoskeletal and connective tissue disorders
Endocrine and metabolic diseases
Genetic and chromosomal diseases
Accidental poisoning and Drowning
Paediatric psychiatric and psychological disorders

- To know communication skills and counseling patients

Time Management :

4th Phase = 6 weeks
6 days / week, 36 days in Total
48 hours in morning
48 hours in evening
24 hours in Outpatient Department
24 hours in Emergency Department

Duration of Placement (2nd Round) fromto
 Total attendancedays, out ofdays

A. History writing :

SL	Case	Date	Supervisor

B. Case Management to be observed

Serial Number	Case Management to be observed	Date	Signature of the teacher
1.	Nutritional: PEM (MAM, SAM), Xerophthalmia ,Rickets		
2.	Cardiovascular: Ventricular septal defect, TOF,HF		
3.	Respiratory: Pneumonia, bronchiolitis, asthma		
4.	Gastrointestinal: diarrhea, hepatitis, chronic liver disease		
5.	Renal: NS, AGN		
6.	Nervous system: Febrile convulsion, meningitis, encephalitis		
7.	Infection: Enteric fever, UTI, Dengue fever, malaria, TB,Kala-azar		
8.	Hematology: ITP, Hemophilia, Thalassemia, Aplastic anemia		
9.	Rheumatology: Rheumatic fever, JIA, HSP, SLE		
10.	Endocrine: Congenital hypothyroidism, DM		
11.	Genetic: Down syndrome, Turner syndrome		
12.	Malignancy: ALL, Lymphoma		
13.	Neonatal: Perinatal asphyxia, LBW, Sepsis, neonatal jaundice		
14.	Accidental poisoning: OPC poisoning, Kerosene poisoning, Corrosive poisoning, Drowning, Snake bite.		

C. Events to be observed:

SL	Events name	Date	Signature
1.	Lumber Puncture		
2.	Bone Marrow Aspiration		
3.	Insertion of Intravenous Line		
4.	Naso-gastric tube introduction		
5.	Per rectal diazepam		
6.	Breast feeding (Positioning & attachment)		
7.	Tepid sponging		
8.	Mantoux test/BCG		

9.	Blood Transfusion/Mobile transfusion		
10.	Collection of blood samples		
11.	Pulse/Temp/Resp recording		
12.	B.P. recording		
13.	Collection of throat swab		
14.	Collection of urine/stool		
15.	Aspiration of Fluid-pleural/abdominal		
16.	Use of Pulse Oxymeter, ambu bag		
17.	Enema Simplex		
18.	Nebulization		
19.	Use of glucometer		
20.	CPR		

D. Clinical classes to attend:

SL No.	Date	Topic	Signature of the teacher	Signature of evening teacher
01		Introduction		
02		History taking		
03		IMCI, IYCF		
04		Developmental Assessment And Growth chart		
05		A child with malnutrition		
06		A child with malnutrition		
07		Diarrhoea in children		
08		A Child with cough & difficult breathing		
09		A Child with cough & difficult breathing		
10		Recurrent wheeze in children		
11		Approach to child with fever and rash		
12		An approach to child with jaundice		
13		A Child with lymphadenopathy		
14		A Child with fever, pallor & hepatosplenomegaly		
15		Management of pallor		
16		Congenital Heart disease & Heart failure		
18		Bleeding disorder in children		
		Bleeding disorder in children		
19		A child with joint swelling		
20		A child with joint swelling		
21		A Child with scanty micturition		
22		A Child with scanty micturition		
23		Convulsion In Children		
24		Convulsion In Children		

25		Accidental Poisoning		
26		Snake bite, Drowning		
27		Breast feeding, IYCF		
28		Low Birth Weight		
29		PNA with neonatal resuscitation		
30		Neonatal Sepsis		
31		Neonatal Jaundice		
32		Vaccination		
33		Assessment		
34		Feedback		

E. Practical works to be done:

SL		Date	Teacher
1.	Pulse/Respiration Rate /Temperature Measurement		
2.	Use of ambu bag		
3.	Measurement of weight, height/Length/OFC & MUAC		
4.	Use of growth chart		
5.	E.N.T examination-auroscope, tongue depressor		

F. Paediatric Emergency management to be observed

Sl		Date	Teacher
1.	Convulsion		
2.	Severe dehydration		
3.	Childhood poisoning Accidents		
4.	Respiratory distress- Acute Asthma		
5.	Heart failure		
6.	Shock		

G. Activities in Child OPD

	Date	Teacher
(1) ORT corner		
i) Preparation ORT		
ii) Monitoring ORT		
iii) Counseling mother		
iv) Preparation of high energy density food (khichuri, halwa)		
(2) Immunization clinic		
i) EPI Vaccination observed/practice OPV		
ii) Counseling witnessed practice		
iii) Cold chain observed		
(3) Shishu Bikashkendra		
(4) Lactation Management Centre		
(identification of problem in breastfeeding, Positioning and attachment)		

H. Activities on Neonatal Ward**Date****Teacher****(1) Examination of Newborn**

i)

.....

ii)

.....

(2) Case management to be observed

i) Perinatal Asphyxia

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ii) Low birth weight

.....

iii) Prematurity

.....

iv) Neonatal jaundice

.....

v) Neonatal infection

.....

Pneumonia

.....

Septicaemia

.....

Umbilical infection

.....

Oral thrush

.....

vi) Essential newborn Care

.....

(3) Events to be observed**Date****Teacher**

1. Hand washing

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2. Breast feeding

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3. Endotracheal intubation/CPR

.....

4. N.G. tube feeding

.....

5. Phototherapy

.....

6. Exchange transfusion

.....

7. Umbilical Catheterization

.....

Marks Obtained (%):

Comment:

Professor

Department of Paediatrics

Registrar

Department of Paediatrics